



# REGIONAL FORENSIC CENTER

## KNOX COUNTY

2761 Sullins Street, Knoxville, TN 37919 • Phone: 865-215-8000 • Fax: 865-215-8001

### AUTOPSY FINAL REPORT

Patient: **VINT, ADAM MICHAEL**

Accession: **250304-631**

Med. Rec. #:

Client: Knox County

Reference #:

DOB/Gender: 01/10/1994 (Age 31) WM

DOD: 03/04/2025 12:03

Physician(s)

Autopsy Date: 03/05/2025 10:30

Reported: 04/08/2025

Authorized by: Darinka Mileusnic-Polchan, M.D., Ph.D.

Relationship to Patient: Knox County Chief Medical Examiner

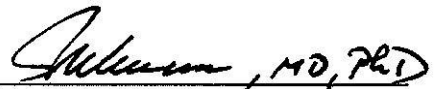
Restrictions: None

Persons Attending Autopsy: Makayla Hyde, M.D., PGY-1, Department of Pathology, UTMCK, and Matthew Hawke, Roane State Paramedic Program

Technician: Katherine Arihood

#### Narrative

This 31-year-old white male, Adam Michael Vint, was discovered deceased on the floor of a motel room that he rented for an extended period of time. Mr. Vint had North Carolina Driver's License and had been in the area allegedly working for several months. Drug paraphernalia were present in the room. Based on the scene investigation and the complete autopsy with ancillary studies, the main cause of death of Adam M. Vint is heroin, fentanyl, xylazine, and mitragynine intoxication due to intravenous drug abuse. The manner of death is accident.

  
Darinka Mileusnic, MD, PhD

#### Final Anatomic Diagnoses

- I. Intravenous drug abuse
  - A. Multiple needle punctures of bilateral upper extremities
  - B. Pulmonary congestion and edema, severe
    1. Underlying respiratory bronchiolitis, multifocal, bilateral, chronic, mild
  - C. Hepatitis C, chronic
    1. Lymphocytic portal triaditis, diffuse
    2. Hepatic steatosis, moderate
  - D. Cerebral edema, severe
- II. Hypertensive cardiovascular disease
  - A. Cardiomegaly, mild
  - B. Atherosclerosis of proximal left anterior descending coronary artery, focal, mild

III. Hydrocele, bilateral

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**Gross Description**

**EXTERNAL EXAMINATION**

The decedent is received wearing a pair of black pants with a yellow rope belt securing the waist, a green-blue T-shirt, a gray sock on the left foot, and an off-white and black sock on the right foot. The front of the pants demonstrates multiple bleached spots. In the left nipple, there is a black metal barbell. In the right nipple, there is a yellow metal nipple ring. Received with the decedent, there is a North Carolina Driver's License.

The body is that of a well-developed and well-nourished white male who appears to be older than the stated age of 31 years. The body measures 72 inches in length and weighs 221 pounds (body mass index 29.97 kilograms/meter<sup>2</sup>).

The body is cold to touch. Rigor mortis is present to an equal extent in all large joints. Postmortem lividity is fixed in the anterior dependent portions of the body with early development of Tardieu spots and a hand imprint or blanched outline over the right anterior thoracic and abdominal area.

The scalp hair is blonde and short and displays top of the head male balding pattern. There is a blonde short mustache and a beard with sideburns.

The eyes are closed. The eyelids are mildly puffy. The irides are blue. The corneae are cloudy. The pupils are fixed centrally and appear symmetrical. The sclerae and conjunctivae are congested with no evidence of petechiae.

On the right lower forehead and over the malar region of the right cheek, there are oval to slightly irregular pressure abrasions with no associated vital reaction, 0.8 inch and 2.0 inches in the greatest dimension, respectively. Inferior to the lower abrasion, there are minor clustered pinpoint irregular superficial lesions consistent with insect activity.

The skeleton of the nose is intact. The external ears display no evidence of trauma. Dark mucus-rich discharge admixed with coffee-ground material covers the lower face.

The lips and frenula are atraumatic. The teeth are natural and in good repair, although stained with coffee-ground material.

The neck is not hypermobile on external manipulation. The chest is symmetrical. The abdomen is slightly protuberant. The external genitalia are of a normal adult circumcised male with no evidence of trauma.

The upper extremities are symmetrical. On the anteromedial aspect of the proximal right forearm, there are three fresh-appearing needle punctures aligned longitudinally. In the left antecubital fossa, there is a 1.5 inch healing lesion with a superimposed needle puncture and red-blue ecchymosis. Between the knuckles for the third, fourth and fifth digits of the dorsal left hand, there are irregular elongated abrasions with no associated vital reaction consistent with insect activity. The fingernails are short and soiled.

The back and buttocks are without special note. The anus is atraumatic.

The lower extremities are symmetrical. The toenails are short and soiled.

**IDENTIFYING MARKS:**

1. On the anterior right thigh, there are mostly professional tattoos consisting of a praying mantis, dumpster fire and

a sword accompanied by symbols.

2. On the upper back, there is a polychromatic tattooed symbol containing an all-seeing eye.

## INTERNAL EXAMINATION

**BODY CAVITIES:** The body is entered by a Y-shaped incision. Most of the organs are present in their ordinary anatomic positions and present their ordinary anatomic relationships. The subcutaneous fat measures 1.5 centimeters at the level of the nipples and 4.0 centimeters at the level of the umbilicus. The viscera is congested.

**NECK ORGANS AND TONGUE:** The anterior muscles of the neck reveal no evidence of hemorrhage. The cartilages of the larynx are intact. The hyoid bone is intact. The lumen of the larynx is free of content. Examination of the tongue demonstrates no injuries.

**RESPIRATORY SYSTEM:** The trachea and bronchi are lined by blood-stained edema fluid. The right lung weighs 1,150 grams. The left lung weighs 1,140 grams. The lungs are purple-black with abundant diffuse subpleural anthracosis. On cut section, the lung parenchyma demonstrates diffuse congestion and generalized severe edema. No granulomas or other nodules are palpable in either lung. The pulmonary arteries are unremarkable.

**CARDIOVASCULAR SYSTEM:** The heart is mildly enlarged and globoid and weighs 440 grams. The epicardium is smooth and glistening and the epicardial fat is adequate. The superior and inferior venae cavae return to the right atrium. The pulmonary veins return to the back of the left atrium in a normal fashion and distribution. On sectioning of the heart, no significant congenital or valvular anomalies are noted. The valve circumferences are as follows: tricuspid 14.0 centimeters, pulmonic 8.0 centimeters, mitral 11.5 centimeters, and aortic 7.0 centimeters. The myocardium is red-brown and uniform. The thicknesses of the right free ventricular wall, the left free ventricular wall, and the interventricular septum are 0.3 centimeter, 1.3 centimeters, and 1.4 centimeters, respectively. The coronary ostia are present in their ordinary anatomic locations. The coronary arteries pursue their ordinary anatomic course and demonstrate focal mild atherosclerosis with less than 25% narrowing of the very proximal segment of the left anterior descending coronary artery. The remaining coronary arteries are fully patent. The coronary circulation is right dominant. The great vessels at the base of the heart are unremarkable. The intima of the aorta demonstrates mild yellow stippling and streaking.

**GASTROINTESTINAL SYSTEM:** The esophagus is without special note. The stomach contains approximately 600 milliliters of brown liquid chyme. The mucosa of the stomach demonstrates diffusely mildly flattened rugae and underlying diffuse cobblestone appearance. The duodenum, small and large intestines are without special note. The appendix is present and shows no pathologic changes.

**HEPATOBIILIARY SYSTEM:** The liver is enlarged and of the usual shape and weighs 2,480 grams. The hepatic capsule is smooth and transparent. The margins of the liver are rounded. On cut section, the hepatic parenchyma is brown-red and diffusely congested. The gallbladder and biliary tract pursue their ordinary anatomic course and demonstrate no pathologic changes. The gallbladder contains approximately 10 milliliters of brown-yellow bile.

**HEMOLYMPHATIC SYSTEM:** The spleen is of the usual shape and size and weighs 220 grams. The splenic capsule is smooth and transparent. On cut section, the splenic parenchyma is purple-blue and mildly congested with inconspicuous corpuscles. No abnormal lymphadenopathy is noted.

**GENITOURINARY SYSTEM:** The kidneys are of the usual shape and size. The right kidney weighs 160 grams. The left kidney weighs 170 grams. The renal capsules are adhered to the underlying cortex. The kidneys are red-brown with diffusely finely granular cortical surfaces. On cut section, the renal parenchyma is red-brown and mildly congested with well-demarcated corticomedullary junctions. The renal calyces, pelves, ureters, and the urinary bladder with prostate demonstrate no abnormalities. The urinary bladder contains approximately 125 milliliters of dark yellow urine. The bilateral testes are descended in their respective scrotal sacs and surrounded by clear serous hydroceles, right greater than left.

NEUROENDOCRINE SYSTEM: The pituitary and thyroid glands are without special note. The adrenal glands and pancreas are autolyzing. The pancreas weighs approximately 190 grams.

CENTRAL NERVOUS SYSTEM: The scalp displays no lacerations or hematomas. On reflecting the scalp, there is no subgaleal hemorrhage. The skull is intact. On entering the cranial cavity, there is no evidence of either subdural or epidural hematoma. The leptomeninges are translucent and congested. The brain weighs 1,670 grams. There is severe brain edema manifested by flattening of the gyri, uncal notching, and cerebellar tonsillar herniation. The blood vessels at the base of the brain show no pathologic changes. Serial coronal sections of the brain reveal no focal areas of pathologic change. Horizontal sectioning of the cerebellum and the brainstem does not reveal abnormalities. The dural sinuses contain clotted blood. The cervical spinal cord is unremarkable.

MUSCULOSKELETAL SYSTEM: All ribs, long bones, and vertebrae are intact to palpation.

ADDITIONAL STUDIES:

1. Chemstrip® analysis of the vitreous humor is negative for either ketones or glucose.
2. Samples of blood and urine are sent for toxicologic analysis.
3. Samples of blood, urine, vitreous humor, bile, and gastric content are saved.
4. A DNA blood card is prepared and saved with the case file.
5. Multiple histologic sections are taken for microscopic studies.
6. Organ and tissue samples are fixed in formalin and saved.

**Microscopic Description**

SLIDE KEY:

- A1) Heart, left ventricle, interventricular septum, and right ventricle
- A2) Right lung, lower lobe, and left lung, upper lobe
- A3) Liver and right kidney
- A4) Left hippocampus and right basal ganglia
- A5) Right hippocampus and pituitary gland

Comment: The significant histopathologic findings are listed under the final anatomic diagnoses.

SG



## NMS Labs

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Robert A. Middleberg, PhD, F-ABFT, DABCC-TC, Laboratory Director

### Toxicology Report

Report Issued 03/19/2025 11:18

To: 10511

Knox County Medical Examiner's Office  
2761 Sullins Street

Knoxville, TN 37919

Patient Name Vint, Adam M

Patient ID 250304-631

Chain 250304-631

DOB 01/10/1994

Sex Male

Workorder 25102175

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### Positive Findings:

| Analyte                     | Result      | Units  | Matrix Source       |
|-----------------------------|-------------|--------|---------------------|
| Caffeine                    | Presump Pos | mcg/mL | 001 - Femoral Blood |
| Cotinine                    | Presump Pos | ng/mL  | 001 - Femoral Blood |
| Nicotine                    | Presump Pos | ng/mL  | 001 - Femoral Blood |
| Morphine - Free             | 100         | ng/mL  | 001 - Femoral Blood |
| 6-Monoacetylmorphine - Free | 2.1         | ng/mL  | 001 - Femoral Blood |
| Lamotrigine                 | 0.60        | mcg/mL | 001 - Femoral Blood |
| Xylazine                    | 15          | ng/mL  | 001 - Femoral Blood |
| Delta-9 THC                 | 1.2         | ng/mL  | 001 - Femoral Blood |
| Norfluoxetine               | 200         | ng/mL  | 001 - Femoral Blood |
| Fluoxetine                  | 230         | ng/mL  | 001 - Femoral Blood |
| Fentanyl                    | 17          | ng/mL  | 001 - Femoral Blood |
| Norfentanyl                 | 0.99        | ng/mL  | 001 - Femoral Blood |
| 4-ANPP                      | 2.5         | ng/mL  | 001 - Femoral Blood |
| Mitragynine                 | 310         | ng/mL  | 001 - Femoral Blood |

See Detailed Findings section for additional information

Agency Case Number: 250304-631

### Testing Requested:

| Test  | Test Name                              |
|-------|----------------------------------------|
| 8052B | Postmortem, Expanded, Blood (Forensic) |

### Specimens Received:

| ID  | Tube/Container          | Volume/<br>Mass | Collection<br>Date/Time | Matrix Source | Labeled As |
|-----|-------------------------|-----------------|-------------------------|---------------|------------|
| 001 | Gray Stopper Glass Tube | 7.75 mL         | 03/05/2025 11:25        | Femoral Blood | 250304-631 |
| 002 | Gray Stopper Glass Tube | 7.75 mL         | 03/05/2025 11:25        | Femoral Blood | 250304-631 |

All sample volumes/weights are approximations.

Specimens received on 03/06/2025.

**Detailed Findings:**

| Analysis and Comments                                                                           | Result      | Units  | Rpt. Limit | Specimen Source     | Analysis By |
|-------------------------------------------------------------------------------------------------|-------------|--------|------------|---------------------|-------------|
| Caffeine                                                                                        | Presump Pos | mcg/mL | 0.20       | 001 - Femoral Blood | LC/TOF-MS   |
| This test is an unconfirmed screen. Confirmation by a more definitive technique is recommended. |             |        |            |                     |             |
| Cotinine                                                                                        | Presump Pos | ng/mL  | 200        | 001 - Femoral Blood | LC/TOF-MS   |
| This test is an unconfirmed screen. Confirmation by a more definitive technique is recommended. |             |        |            |                     |             |
| Nicotine                                                                                        | Presump Pos | ng/mL  | 100        | 001 - Femoral Blood | LC/TOF-MS   |
| This test is an unconfirmed screen. Confirmation by a more definitive technique is recommended. |             |        |            |                     |             |
| Morphine - Free                                                                                 | 100         | ng/mL  | 5.0        | 001 - Femoral Blood | LC-MS/MS    |
| 6-Monoacetylmorphine - Free                                                                     | 2.1         | ng/mL  | 1.0        | 001 - Femoral Blood | LC-MS/MS    |
| Lamotrigine                                                                                     | 0.60        | mcg/mL | 0.20       | 001 - Femoral Blood | LC-MS/MS    |
| Xylazine                                                                                        | 15          | ng/mL  | 5.0        | 001 - Femoral Blood | LC-MS/MS    |
| Delta-9 THC                                                                                     | 1.2         | ng/mL  | 0.50       | 001 - Femoral Blood | LC-MS/MS    |
| Norfluoxetine                                                                                   | 200         | ng/mL  | 20         | 001 - Femoral Blood | GC/MS       |
| Fluoxetine                                                                                      | 230         | ng/mL  | 20         | 001 - Femoral Blood | GC/MS       |
| Fentanyl                                                                                        | 17          | ng/mL  | 1.0        | 001 - Femoral Blood | LC-MS/MS    |
| Norfentanyl                                                                                     | 0.99        | ng/mL  | 0.40       | 001 - Femoral Blood | LC-MS/MS    |
| 4-ANPP                                                                                          | 2.5         | ng/mL  | 0.20       | 001 - Femoral Blood | LC-MS/MS    |
| Mitragynine                                                                                     | 310         | ng/mL  | 5.0        | 001 - Femoral Blood | LC-MS/MS    |

**Examination of the specimen(s) submitted did not reveal any reportable findings by procedure(s) outlined in the accompanying Analysis Summary, other than those listed above. Interpretation of reported findings should be based on the totality of available case information. Reference information is not case-specific but is provided as a general guide.**

**Reference Comments:**

1. 4-ANPP (Despropionyl fentanyl) - Femoral Blood:

4-ANPP (despropionylfentanyl) is a precursor chemical used in the production of fentanyl/fentanyl related analytes and is also a fentanyl metabolite and may be a metabolite of other fentanyl-related analytes. It is considered to be pharmacologically weak.

2. 6-Monoacetylmorphine - Free (6-MAM; Heroin Metabolite) - Femoral Blood:

6-monoacetylmorphine (6-MAM) is the 6-monoacetylated form of morphine, which is pharmacologically active. When present, it is generally indicative of heroin (diacetylmorphine) use. 6-MAM has also been reported to occur as an artifact in samples with unusually high blood morphine concentrations.

A healthy man administered 12 mg heroin intravenously achieved peak blood concentrations at two minutes post injection of 150 ng/mL of 6-MAM and 44 ng/mL of morphine, which declined with half-lives of 7 minutes and 33 minutes, respectively.

Eight subjects who died within fifteen minutes of heroin administration had postmortem blood 6-MAM concentrations averaging 19 ng/mL with a range from less than 1.0 to 82 ng/mL.

**Reference Comments:**

## 3. Delta-9 THC (Active Ingredient of Marijuana) - Femoral Blood:

Delta-9 THC is the principle psychoactive ingredient of marijuana (cannabis, hashish). It is also the active component of the prescription medication Marinol®. Marijuana use causes relaxation, distorted perception, euphoria and feelings of well being, along with confusion, dizziness, somnolence, ataxia, speech difficulties, lethargy and muscular weakness.

After smoking a user-preferred 300 mcg/kg dose average plasma THC concentrations at 35 minutes were reported at 16.1 (range 4.7-30.9) ng/mL, and had declined to 1.5 (range 0.4-3.2) ng/mL after 190 minutes. Usual peak levels in serum for 1.75% or 3.55% THC marijuana cigarettes: 50-270 ng/mL at 6 to 9 minutes after beginning smoking, decreasing to less than 5 ng/mL by 2 hrs. Whole blood THC concentrations are typically half those in a corresponding plasma sample.

## 4. Fentanyl (Duragesic®; Sublimaze®) - Femoral Blood:

Fentanyl is a prescription opioid commonly used as an anesthetic/analgesic. It is reported to be 80 to 200 times as potent as morphine and has a rapid onset of action as well as addictive properties. Illicit fentanyl is readily available due to low production cost and its high potency. It is often sold as heroin and is commonly found in combination with other illicit drugs. Signs associated with fentanyl toxicity include severe respiratory depression, muscle rigidity, seizures, hypotension, coma and death.

When used clinically as a transdermal preparation (25-100 mcg/hour patch), serum fentanyl concentrations up to 3.8 ng/mL have been reported within 24 hours. Following removal of the patch, serum fentanyl concentrations are reported to decrease with a mean elimination half-life of 17 hours (range, 13-22 hours). The mean peak plasma serum fentanyl concentration in adults given an 800 mcg oral transmucosal fentanyl preparation over 15 minutes is reported at 2.1 ng/mL (range, 1.4-3.0 ng/mL) at approximately 0.40 hours.

It is reported that patients lost consciousness at mean plasma levels of fentanyl of 34 ng/mL when infused with 75 mcg/Kg over a 15 min period; peak plasma levels averaged 50 ng/mL. In fatalities from fentanyl, blood concentrations are variable and have been reported as low as 3 ng/mL. Postmortem blood fentanyl concentrations ranged from 0.30-110 ng/mL (median 11 ng/mL) in 301 femoral blood specimens obtained from accidental drug overdose death investigations. These concentrations ranged from 9.7-41.3 ng/mL (median 17.2 ng/mL) in 7 fentanyl only cases in another published case series.

The blood to serum or plasma ratio is 0.8-1.0

## 5. Fluoxetine (Prozac®) - Femoral Blood:

Fluoxetine is a chemically-atypical antidepressant used to help control major depressive disorders. Norfluoxetine, the major metabolite of fluoxetine, is also active pharmacologically. Recommended daily doses range between 20 to 80 mg.

Following a single 40 mg dose, reported peak plasma levels were between 20-60 ng/mL after 6 to 8 hr. Chronic daily doses of 40 mg for 1 month produced reported plasma concentrations ranging from 90-300 ng/mL for fluoxetine and 70-300 ng/mL for norfluoxetine. There is, however, no clear relationship between plasma concentrations of fluoxetine and/or norfluoxetine and efficacy.

Toxicity with fluoxetine is not routinely observed at pre-mortem combined concentrations of fluoxetine and norfluoxetine below 2000 ng/mL. Concentrations much greater than 2000 ng/mL are not necessarily fatal. There have been reports of survived overdose involving fluoxetine with combined blood or plasma concentrations of parent compound and metabolite over 4000 ng/mL. In deaths attributable to fluoxetine overdose, reported blood or plasma combined concentrations range from 2000-11000 ng/mL.

**Reference Comments:****6. Lamotrigine (Lamictal®) - Femoral Blood:**

Lamotrigine is a drug used in the treatment of epilepsy, often in combination with other anticonvulsant drugs. Adults are often started on doses as low as 25 mg every other day and then gradually increased to a maintenance dose that may range between 100-700 mg/day.

A therapeutic range of 3-14 mcg/mL has been proposed, but pharmacokinetics can vary with the administration of other antiepileptic drugs. Adverse reactions associated with therapy include skin rash, dizziness, headache, somnolence, ataxia, blurred vision, nausea and vomiting.

The blood to serum or plasma ratio is unknown.

A lamotrigine concentration of 54 mcg/mL in femoral blood has been reported in a fatality believed to be due to lamotrigine.

**7. Mitragynine (Kratom) - Femoral Blood:**

Mitragynine is an alkaloid found in the plant Kratom which originates from Asia and is considered an atypical opioid agonist. The leaves of the plant are traditionally chewed or brewed into a tea, however, Kratom powders and leaf extracts are also sold and may be subject to abuse. The effects of Kratom are dose dependent, with lower doses resulting in stimulant-type effects and higher doses resulting in opioid-type effects. Reported adverse effects include seizures, psychosis, and liver toxicity.

Over a 27 month-period, mitragynine was reported in 1001 blood specimens submitted during the course of medicolegal investigations; concentrations ranged from 5.6-29,000 ng/mL. In three cases which ruled mitragynine as the sole drug in the cause of death, postmortem peripheral blood concentrations ranged from 1590-3420 ng/mL.

**8. Morphine - Free (Codeine Metabolite) - Femoral Blood:**

Morphine (Duramorph, Roxanol, MS-Contin) is a DEA Schedule II opiate narcotic analgesic. It can be a metabolite or breakdown product of codeine and heroin. If found together with 6-monoacetylmorphine (6-MAM), likely source is heroin. A large portion of the morphine may be conjugated; the portion not conjugated is termed 'free morphine', the active biologic agent which is a powerful painkilling drug whose diverse effects that may include analgesia, drowsiness, nausea and respiratory depression. Hydromorphone is a reported metabolite of morphine.

Morphine peak serum concentrations occur within 10 to 20 minutes of a 10 mg/70 kg intramuscular dose, average 60 ng/mL 30 minutes following administration. IV administration of 10 mg/70 kg, average 80 ng/mL after 30 minutes. Chronic pain patients receiving an average of 90 mg (range 20-1460) daily oral morphine had average serum concentrations of 73 ng/mL (range 13-710) morphine. In 15 cases where cause of death was attributed to opiate toxicity (heroin, morphine or both), free morphine concentrations were 0-3700 ng/mL (average 420 +/- 940). In comparison, in cases where COD was unrelated to opiates (n=20) free morphine was 0-850 ng/mL (average 90 +/- 200). The ratio of whole blood concentration to serum or plasma concentration is approximately one. In a population of 676 drivers arrested for driving under the influence, Morphine concentrations ranged from 1.25-1290 ng/mL, with an average of 52 ng/mL.

Following excessive opiate use, pupils are typically constricted and unreactive to light. Pulse and blood pressure, and body temperature can be lowered. Psychomotor impairment is generally present, with increased body sway, and poor performance in divided attention tests. Users are sometimes described as 'on the nod', falling asleep in the middle of conversations or at inappropriate times. Tolerance can develop to the effects of opiates, and more experienced users are less susceptible to the impairing effects.

**9. Norfentanyl (Fentanyl Metabolite) - Femoral Blood:**

Norfentanyl is the primary inactive metabolite of the synthetic narcotic analgesic fentanyl.

**10. Norfluoxetine (Fluoxetine Metabolite) - Femoral Blood:**

Daily therapy with 40 mg fluoxetine/day: Steady-state concentration at 4 to 8 hr after dosing ranges from 72-258 ng/mL in serum.

### Reference Comments:

11. Xylazine (Rompun®) - Femoral Blood:

Xylazine is a veterinary sedative, analgesic, and anesthetic in large animals (e.g., cattle) that is also found as an adulterant in illicit drugs such as fentanyl. It is not approved for human use. Adverse effects of xylazine exposure include drowsiness/lethargy, bradycardia, hypotension, hypertension, slurred speech, respiratory depression, coma, and death.

Blood concentrations in approximately 3000 postmortem cases and 130 driving under the influence of drugs cases were 5.0-1700 ng/mL (Mean: 34 +/- 1 ng/mL) and 5.1-450 ng/mL (Mean: 36 +/- 4 ng/mL), respectively; 99% of cases contained an opioid, primarily fentanyl. Two postmortem cases with xylazine concentrations of 9100 and 11000 ng/mL were determined to be intentional xylazine overdoses. Serum/plasma concentrations in non-fatal exposures were 30-1500 ng/mL.

The blood to serum/plasma ratio of xylazine is unknown.

### Sample Comments:

001 Physician/Pathologist Name: Darinka Mileusnic

Unless alternate arrangements are made by you, the remainder of the submitted specimens will be discarded one (1) year from the date of this report; and generated data will be discarded five (5) years from the date the analyses were performed.

Workorder 25102175 was electronically signed on 03/19/2025 10:28 by:



Meaghan R. Hessler, M.S.F.S., D-ABFT-FT  
Forensic Toxicologist

### Analysis Summary and Reporting Limits:

The following test(s) were performed for this case; the scope of each test includes the analyte(s) listed along with the associated reporting limit(s). The reporting limit is the lowest concentration of the analyte that will be reported as positive. Only results that meet reporting criteria at or above the reporting limit appear in the Positive Findings section of the report.

Test 50016B - Opiates - Free (Unconjugated) Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

| Analyte                            | Rpt. Limit | Analyte            | Rpt. Limit |
|------------------------------------|------------|--------------------|------------|
| 6-Monoacetylmorphine - Free        | 1.0 ng/mL  | Hydrocodone - Free | 5.0 ng/mL  |
| Codeine - Free                     | 5.0 ng/mL  | Morphine - Free    | 5.0 ng/mL  |
| Dihydrocodeine / Hydrocodol - Free | 5.0 ng/mL  | Oxycodone - Free   | 5.0 ng/mL  |

Test 52059B - Lamotrigine Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

| Analyte     | Rpt. Limit  | Analyte | Rpt. Limit |
|-------------|-------------|---------|------------|
| Lamotrigine | 0.20 mcg/mL |         |            |

Test 52135B - Xylazine Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

## Analysis Summary and Reporting Limits:

| <u>Analyte</u> | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|----------------|-------------------|----------------|-------------------|
| Xylazine       | 5.0 ng/mL         |                |                   |

Test 52198B - Cannabinoids Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

| <u>Analyte</u>         | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|------------------------|-------------------|----------------|-------------------|
| 11-Hydroxy Delta-9 THC | 1.0 ng/mL         | Delta-9 THC    | 0.50 ng/mL        |
| Delta-9 Carboxy THC    | 5.0 ng/mL         |                |                   |

Test 52287B - Fluoxetine and Metabolite Confirmation, Blood: 001 - Femoral Blood

-Analysis by Gas Chromatography/Mass Spectrometry (GC/MS) for:

| <u>Analyte</u> | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|----------------|-------------------|----------------|-------------------|
| Fluoxetine     | 20 ng/mL          | Norfluoxetine  | 20 ng/mL          |

Test 52486B - Fentanyl and 4-ANPP Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

| <u>Analyte</u>  | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|-----------------|-------------------|----------------|-------------------|
| 4-ANPP          | 0.20 ng/mL        | Fentanyl       | 1.0 ng/mL         |
| Acetyl Fentanyl | 0.20 ng/mL        | Norfentanyl    | 0.40 ng/mL        |

Test 52495B - Mitragynine Confirmation, Blood: 001 - Femoral Blood

-Analysis by High Performance Liquid Chromatography/ Tandem Mass Spectrometry (LC-MS/MS) for:

| <u>Analyte</u> | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|----------------|-------------------|----------------|-------------------|
| Mitragynine    | 5.0 ng/mL         |                |                   |

Test 8052B - Postmortem, Expanded, Blood (Forensic): 001 - Femoral Blood

-Analysis by Enzyme-Linked Immunosorbent Assay (ELISA) for:

| <u>Analyte</u> | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|----------------|-------------------|----------------|-------------------|
| Barbiturates   | 0.040 mcg/mL      | Gabapentin     | 5.0 mcg/mL        |
| Cannabinoids   | 10 ng/mL          | Salicylates    | 120 mcg/mL        |

-Analysis by Headspace Gas Chromatography (GC) for:

| <u>Analyte</u> | <u>Rpt. Limit</u> | <u>Analyte</u> | <u>Rpt. Limit</u> |
|----------------|-------------------|----------------|-------------------|
| Acetone        | 5.0 mg/dL         | Isopropanol    | 5.0 mg/dL         |
| Ethanol        | 10 mg/dL          | Methanol       | 10 mg/dL          |

-Analysis by High Performance Liquid Chromatography/Time of Flight-Mass Spectrometry (LC/TOF-MS) for: The following is a general list of analyte classes included in this screen. The detection of any specific analyte is concentration-dependent. Note, not all known analytes in each specified analyte class are included. Some specific analytes outside of these classes are also included. For a detailed list of all analytes and reporting limits, please contact NMS Labs. Amphetamines, Anticonvulsants, Antidepressants, Antihistamines, Antipsychotics, Benzodiazepines, CNS Stimulants, Cocaine and Metabolites, Hallucinogens, Hypnotosedatives, Muscle Relaxants, Non-Steroidal Anti-Inflammatory Agents, Opiates and Opioids.



## KNOX COUNTY REGIONAL FORENSIC CENTER

2761 SULLINS STREET, KNOXVILLE, TN 37919  
FAX: 865-215-8001 EMAIL: INV.KNOXRFC@KNOXCOUNTY.ORG

### Demographic Information

|                       |                           |                   |                   |
|-----------------------|---------------------------|-------------------|-------------------|
| <b>Case number:</b>   | 250304-631                | <b>Address 1:</b> | 31 Blanton Street |
| <b>Death County:</b>  | Knox                      | <b>Address 2:</b> |                   |
| <b>Last name:</b>     | Vint                      | <b>City:</b>      | Asheville         |
| <b>First name:</b>    | Adam                      | <b>State:</b>     | North Carolina    |
| <b>Middle name:</b>   | Michael                   | <b>County:</b>    | Buncombe          |
| <b>Race:</b>          | White                     | <b>Zip:</b>       | 28801             |
| <b>Sex:</b>           | Male                      |                   |                   |
| <b>Age:</b>           | 31 years 1 months 22 days |                   |                   |
| <b>Date of birth:</b> | 01/10/1994                |                   |                   |

### Background Information

|                                  |                             |                           |                  |
|----------------------------------|-----------------------------|---------------------------|------------------|
| <b>Investigator notified:</b>    | 2025-03-04 11:30            | <b>Date LKA:</b>          | 03/03/2025       |
| <b>Notified by:</b>              | KPD Dispatch                | <b>Time LKA:</b>          | 21:19            |
| <b>Facility:</b>                 | East TN RFC (Knoxville)     | <b>Found dead:</b>        | 03/04/2025 11:15 |
| <b>Authorized by:</b>            | Megan Sharpe                | <b>911 call received:</b> | 2025-03-04 11:19 |
| <b>ETA at Facility/FH:</b>       |                             | <b>Date of death:</b>     | 03/04/2025 On    |
| <b>Scene investigated ME:</b>    | Yes                         | <b>Time of death:</b>     | 12:03            |
| <b>Scene investigated by LE:</b> | Yes                         | <b>Pronounced:</b>        | 03/04/2025 12:03 |
| <b>Officer:</b>                  | M. Gentry, P. Tonkin        | <b>Scene arrival:</b>     | 2025-03-04 12:03 |
| <b>Agency:</b>                   | Knoxville Police Department | <b>Scene departure:</b>   | 2025-03-04 12:54 |
| <b>Incident #:</b>               | 25-006875                   |                           |                  |

### Next of Kin

|                                |                             |
|--------------------------------|-----------------------------|
| <b>Name:</b> Vint-Rene, Laurie | <b>Relationship:</b> Mother |
|--------------------------------|-----------------------------|

### Cause and Manner of Death

**Reason for reporting:** Natural/Sudden/Unexpected Death  
**Disposition:** Sent to Autopsy Facility

I hereby declare that after receiving notice of death described herein, I conducted an investigation regarding the cause of death in accordance with Section 38-7-109 Tennessee Code Annotated and that the information contained herein regarding such death is true and correct to the best of my knowledge and belief.

**Authorization:** ME/C ordered

**DC signed by:** Office of ME/C

**RFC MDI:**

**DC signer:** Darinka Mileusnic

In accordance with Tennessee Code Annotated 38-7-106, the accompanying body of Vint, Adam is the subject of an investigation by the medical examiner.

Authorizing Signature of Medical Examiner or Delegated Investigator:

## Narrative Summary of Circumstances Surrounding Death

On March 4, 2025 at approximately 1130 hours, the Regional Forensic Center was contacted by the Knoxville Police Department Dispatch. Dispatchers were calling to notify the Regional Forensic Center that officers were out with a deceased male in room 204 of the Econo Lodge located at 5605 Merchants Center Boulevard in Knoxville, Tennessee.

Upon arrival to the scene, RFC Investigator M. Sharpe was met by Sergeants M. Gentry and P. Tonkin with the Knoxville Police Department (KPD). The decedent was reported to be Adam Michael Vint, D.O.B. 01/10/1994, confirmed by a North Carolina driver's license located by law enforcement prior to Inv. Sharpe's arrival. Several KPD patrol officers were also on scene.

The following narrative is a summary of the information that was obtained from law enforcement and Investigator Sharpe's scene findings.

The decedent was last seen alive on 03/03/2025 at 2119 hours by front desk staff when the decedent checked in at the above-stated address. The decedent reportedly was alone when he checked in. Management went to the room on 03/04/2025 around 1115 hours as the decedent had missed his checkout time. Staff used a key to enter the room and found the decedent unresponsive on the floor at the foot of the bed closer to the door. E-911 was contacted at 1119 hours for assistance. Upon the arrival of AMR Ambulance Service, the decedent was found without obvious signs of life. There were no attempts made by EMS to resuscitate the decedent.

Death was officially pronounced upon Investigator Sharpe's arrival to the scene at 1203 hours. The room had two beds and appeared clean. Ambient temperature was 64 degrees Fahrenheit per independent thermometer. The decedent was found on the floor at the foot of the bed closer to the door. The decedent was in a recumbent position on his right side with his face turned toward the floor. The decedent was clothed in a t-shirt, pants, and socks. A bag of capped and sealed syringes was found on the dresser. A baggie with multiple rock-like substances, a can with residue, and a syringe cap were found on the bathroom counter. Two capped syringes were found in the trashcan near the bathroom. The decedent's wallet, cell phone, laptop, and bags were all left in the room to be confiscated by law enforcement along with the suspected drugs and paraphernalia.

The decedent was a 31-year-old White male who was cool to the touch. Rigor mortis was present throughout and firm. Livor mortis was present on the anterior and right lateral aspects, which was consistent with position found, light purple in color, and blanched with firm pressure. The corneas were clear with congestion noted to the sclerae. The nares were congested with mucus and brown-tinged fluid. The dentition and oral airway were unobservable due to clenching in the jaw. The abdomen was firm to palpation. A tattoo was visible on the upper back. The arms, chest, abdomen, genitalia, back of the head, back, and buttocks appeared atraumatic. The remainder of the body was unobservable due to clenching in the jaw. Photographs of the scene and body were taken for review.

A query of the Controlled Substance Monitoring Database yielded a positive result. The decedent received two buprenorphine tablets for one day on 06/20/2024. No medications were found on scene.

There were no concerns for trauma or foul play. Past medical history obtained from medical records included drug use, tobacco use, and bipolar disorder. Medical records were requested from area facilities for review.

Due to the possibility of an overdose, the decedent was transported by Knoxville Forensic Transport to the Regional Forensic Center for an examination by a forensic pathologist. Prior to transport, Investigator Sharpe placed a green identification band on the decedent's left ankle. The body recovery bag was sealed with seal #0908105, a secondary green identification band, and the decedent's identification.

The decedent's next of kin was unknown at the time of the report.

Law enforcement had no special requests. Sergeant Gentry notified the KPD Drug Task Force of the death while on scene.

*The investigative information provided in the above report is subject to change as new information becomes available.*